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COMMENTARY

Despite increased funding, transit ridership hasn't returned in most areas

States where transit continued to support local, all-day travel generally saw steadier post-pandemic recoveries than those focused primarily on peak-hour commuting into downtown job centers.



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Before 2020, much of U.S. transit planning, especially in metropolitan areas with long-established rail networks, was built around the weekday commute. Heavy rail, light-rail and downtown-oriented commuter rail systems were designed to move large volumes of workers into central business districts during peak hours, and investment decisions often reflected that priority.

Most bus systems followed a different logic. In smaller or more dispersed metropolitan areas, buses were more likely to support local, all-day travel across the metro area, serving non-commute trips such as medical appointments, shopping, and service-sector work, because bus networks are less constrained by fixed infrastructure. They have historically played a broader role in meeting everyday mobility needs beyond the traditional rush-hour commute. This model worked well for decades until the COVID-19 pandemic disrupted the assumptions that had long shaped how transit systems were planned and used.

During the pandemic and shutdowns, transit ridership declined rapidly across nearly every state in the United States, leading to significantly reduced fare revenue and placing tremendous financial pressure on transit agencies. In response, Congress approved the largest package of federal transit assistance programs in American history, providing approximately [\\$25 billion](#) through the Coronavirus Aid, Relief, and Economic Security (CARES) Act, \$14 billion via the Coronavirus Response and Relief Supplemental Appropriations Act (CRRSAA), and adding [\\$30.5 billion](#) in the American Rescue Plan for public transit.

At the time, policymakers anticipated that, once offices reopened and daily routines resumed, ridership and transit system performance would recover.

Examining metro transit recovery patterns in select areas since the pandemic

The table below groups major metropolitan transit markets by the percentage of their 2019 ridership that had returned by 2024, using unlinked passenger trips reported to the Federal Transit Administration’s [National Transit Database](#). Each metro is listed with the main transit agency or agencies that operate service in that area. Using metro areas rather than states avoids double-counting riders and helps explain why recovery can differ within a single state. Large urban systems that rely heavily on rail and downtown commuting have often recovered more slowly, while smaller, bus-based systems in other parts of the state have tended to rebound more quickly.

Recovery Category	States
Exceeding 2019 Ridership	Wilmington, DE (DART First State)
Strong Recovery of 2019 Ridership (~85–95%)	Miami–Dade, FL (Miami-Dade Transit); New Orleans, LA (New Orleans RTA); Wichita, KS (Wichita Transit); Sioux Falls, SD (Sioux Area Metro)
Upper-Moderate Recovery of 2019 Ridership (~75–84%)	Salt Lake City / Wasatch Front, UT (Utah Transit Authority); Detroit, MI (DDOT and SMART buses); Des Moines, IA (Des Moines Area Regional Transit Authority); Boise, ID (Valley Regional Transit); Cheyenne and Casper, WY (local bus systems)
Moderate Recovery of 2019 Ridership (~65–74%)	Phoenix, AZ (Valley Metro); Denver, CO (Regional Transportation District); Los Angeles, CA (Los Angeles County Metropolitan Transportation Authority); Chicago, IL (Chicago Transit Authority); Boston, MA (Massachusetts Bay Transportation Authority); Seattle, WA (King County Metro; Sound Transit); Dallas–Fort Worth, TX (Dallas Area Rapid Transit); Portland, OR (TriMet)
Below ~65% Recovery of 2019 Ridership	Atlanta, GA (Metropolitan Atlanta Rapid Transit Authority); Twin Cities, MN (Metro Transit); New York City, NY (MTA New York City Transit); Newark–Jersey City, NJ (NJ Transit; PATH); San Francisco Bay Area (Bay Area Rapid Transit)

What the 2019-2024 transit ridership data show

After five years, the data tell a more complicated story. Although ridership has increased since its low point in 2020–2021, most places have not returned to pre-pandemic ridership levels despite unprecedented federal aid and higher state and local spending.

Some transit systems are close to recovery; a handful have surpassed 2019 ridership, but many remain well below their earlier benchmarks. Across the markets reviewed, 2024 ridership typically falls between roughly two-thirds and three-quarters of 2019 levels, with fewer regions reaching 80–90% of pre-pandemic levels, and only one region, Wilmington, Delaware, exceeding pre-pandemic use in this sample.

The data from 2019 to 2024 point to a clear conclusion: increased short-term federal funding did not consistently lead to higher transit ridership in the areas examined. Even as transit funding rose nationwide, ridership recovery varied widely by state. Pandemic-era policies treated transit ridership losses as a revenue problem to be solved with emergency aid. That approach stabilized transit agency finances but often failed to address whether transit service still matched how people were traveling after the pandemic, even as work patterns, travel behavior, and daily trip-making continued to evolve.

As my Reason Foundation colleague Marc Scribner has noted in his analysis of [post-pandemic transit finances](#), federal relief largely addressed budget shortfalls while ridership across most regions remained well below pre-pandemic levels. The result was financial stability without a corresponding return of ridership.

This distinction matters because it reflects a shift in transit's core challenge. The issue is no longer whether transit agencies can remain solvent. It is whether the service they provide still aligns with how and where people travel.

The transit ridership divide that governments must address

Between 2019 and 2024, states worked with local governments to keep transit service operating through a period of historic ridership loss. Funding levels remained elevated across many systems even as fare revenue collapsed and travel patterns shifted. Yet ridership recovery varied widely from state to state.

Based on data reported to the Federal Transit Administration's [National Transit Database](#), in Georgia, statewide ridership fell from roughly 142.3 million trips in 2019 to 55.3 million in 2021, a 61% decline. By 2024, ridership rebounded to about 80.1 million trips, still approximately 44% below pre-pandemic levels. Colorado experienced a similar trajectory, dropping from 134.2 million trips in 2019 to 66.9 million in 2021 before recovering to 92.0 million trips in 2024, or about 69% of its 2019 ridership.

Large, rail-focused states show an even clearer disconnect between funding and recovery. New York received more than \$25.5 billion in federal transit funding between 2019 and 2024, in addition to substantial state and local support, yet recorded approximately 1.63 billion transit trips in 2024, only about two-thirds of its 2019 total.

New Jersey and Massachusetts followed similar patterns. Despite billions in combined federal and state funding, transit ridership in 2024 reached only about 64% of 2019 levels in New Jersey and roughly 72% in Massachusetts.

By contrast, several more rural, lower-spending, bus-focused states recovered more fully. By 2024, Delaware slightly exceeded its 2019 ridership. Kansas, after experiencing a roughly 50% decline during the pandemic, rebounded to about 87% of its pre-pandemic ridership despite comparatively modest funding levels. Other primarily bus-based states also returned closer to earlier ridership levels, including South Dakota at about 84%, Iowa at about 81%, Michigan at about 80%, and Wyoming at about 83%.

At the same time, several states with comparatively high total transit funding continued to lag in ridership recovery. Minnesota applied roughly \$4.6 billion in combined federal, state, and local transit funding between 2019 and 2024, yet recovered only about 64% of its 2019 transit ridership by 2024.

Similar outcomes appear in Maryland, which applied about \$7.6 billion and recovered roughly 67% of pre-pandemic ridership, Virginia, which used about \$10.9 billion and recovered about 66%, and Pennsylvania, which applied approximately \$30.1 billion and recovered about 65%.

Taken together, these transit ridership patterns suggest that post-pandemic recovery depended less on total spending levels and more on whether transit systems continued to serve practical, everyday travel needs rather than primarily commuter-oriented demand.

Transit ridership recovery followed usefulness, not spending

How well transit recovered after the pandemic depended largely on how systems were designed and who they served. States where transit continued to support local, all-day travel generally saw steadier recoveries than those focused primarily on peak-hour commuting into downtown job centers.

Where transit remained useful for health care workers, service-sector employees, students, and people making everyday trips, ridership returned more quickly.

In contrast, simply restoring pre-pandemic schedules in commuter-focused systems often failed to bring riders back because work and travel patterns had changed.

Decisions by state departments of transportation and transit agencies played a major role in shaping outcomes. Systems with stronger recoveries since the pandemic tended to focus on three actions:

1. Restoring frequent transit service on high-demand corridors.
2. Simplifying transit networks to make routes easier to understand and use.
3. Prioritizing safety and cleanliness to rebuild rider confidence.

Agencies that instead preserved underused routes or maintained legacy service patterns often experienced weaker ridership recoveries, even with substantial funding increases. The key question, then, is not how much funding was provided but how effectively it was used. Ridership remains the clearest measure of value, reflecting whether transit service is frequent, reliable, safe, and aligned with how people travel today.

Going forward, states and transit agencies should prioritize funding based on outcomes

To achieve better returns on future transit spending, states and transit agencies should prioritize funding based on measurable outcomes rather than established practices. What concerns transit riders the most is simple: increased service frequency that gives them options, whether the bus or train shows up when it is supposed to, and if they feel safe riding the system. Operating support for transit systems should be allocated to address these key customer priorities.

On high-ridership routes, transit should run frequently throughout the day to meet demand, not just during morning and evening rush hours. For many urban corridors, that means buses or trains should run every 10 to 15 minutes as a baseline, with more frequent service where ridership supports it. This frequency is the difference between transit being a backup option and being a reliable part of daily life. When service runs this often, people can use it for work, school, medical appointments, and errands without checking a schedule or worrying about missing a trip.

When transit service is infrequent, even motivated riders are forced to plan their day around the timetable, and many simply choose to drive instead. Frequency is not about convenience. It is about whether transit is practical to use at all.

Reliability should be evaluated from the rider's perspective rather than through internal documentation. On scheduled routes, vehicles should depart no more than one minute early and no more than five minutes late. On high-frequency routes, maintaining consistent vehicle spacing is essential to prevent excessive waits caused by vehicle bunching.

Safety should be evaluated based on real-world results rather than internal compliance. Transit riders are concerned with incidents such as assaults, operator injuries, serious collisions, and disorder that can undermine predictability and security or simply make riders feel unsafe. States should link a portion of operating support for transit to measurable improvements in these areas rather than solely to administrative compliance. Transit agencies should be required to demonstrate reductions in serious incidents and injuries.

States should also encourage transit agencies to redesign their networks according to current travel patterns rather than pre-pandemic commuter trends. This redesign should include implementing simpler routes, enhancing all-day service, and reducing low-ridership coverage routes maintained primarily due to historical precedent.

Bus speed and reliability improvements, such as dedicated lanes, signal priority, and optimized stop spacing, should be reserved for corridors with demonstrated or latent demand sufficient to justify reallocating roadway capacity. Exclusive lanes are most cost-effective where buses already carry high passenger volumes or where time savings and frequency improvements are likely to attract significant new ridership.

In areas with low or dispersed demand, dedicating lanes may result in underutilization and inefficient use of limited road space. Therefore, states and transit agencies should prioritize lower-cost reliability measures, including schedule adherence improvements, stop consolidation, running-time adjustments, and targeted signal priority, until demand justifies more intensive interventions.

Currently, many states allocate operating support using legacy formulas based on historical or geographic factors rather than current transit ridership and performance. This approach prioritizes preservation over effectiveness. Public funding should be directed toward transit routes and services that demonstrably enhance riders' mobility.

The objective should be to develop transit systems that attract and serve riders in 2026 and beyond, rather than restoring transit systems to their 2019 state.

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